

Other papulosquamous disorder

Dr eman gomaa

Consultant dermatologist

1. Ptyriasis rosea



• GPPS
Guttate ps
Pr
Plc
2ry syphilis



✓ Ptyriasis rosea

HHV6 _HHV7

- ▶ A self-limited papulosquamous eruption that is occasionally pruritic
- ▶
 - Seen primarily in adolescents and young adults, favoring the trunk and proximal extremities. With female predominance
- ▶
 - Herald patch followed by lesions that are usually oval in shape and oriented with their long axes along Langer cleavage lines
- ▶
 - Less common variants include inverse, vesicular, purpuric, and pustular

► Signs

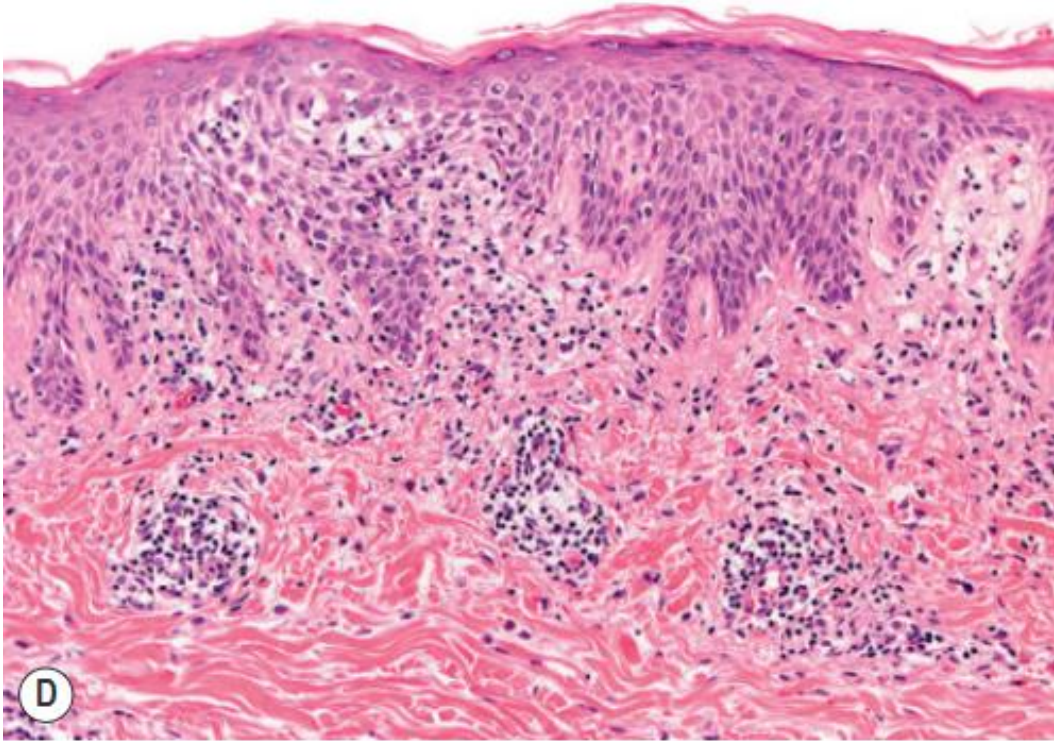
(1) The classic disease

- - Begins with a solitary lesion (herald patch) a few days before onset of a more generalized eruption.
- The initial lesion (herald patch):
 - o A sharply-defined erythematous patch or thin plaque, round or oval, 2 - 6 cm with a collaret of scales at the periphery
- - o Most commonly occur on the trunk (less commonly on neck or upper arm - may appear anywhere on the body).

- ▶ Generalized eruption (days to wks)
- ▶ (lesions look like smaller herald patches)
- ▶ Located mainly on the trunk (front / back), neck and proximal upper limbs,
 - o The long axis of these lesions follows the planes of cleavage. On the posterior trunk, running parallel to the ribs resulting into ‘fir tree’ or “Christma-tree” pattern,
- ▶
 - o Lesions continue to appear in crops for 7 - 10 days.

Varients of PR			
SITE	MORPHOLOGY	HERALD PATCH	DRUG
Inverted Generalized localized	Papular Vesicular Pustular Urticarial Purpuric Lichenoid papules	Only (abortive) Not present	ACE inhibitor B blocker NSAID BCG VACCINE Gold Isotretinoin

histopathology



- ▶ The microscopic features of pityriasis rosea are nonspecific.
- ▶ Small mounds of parakeratosis,
- ▶ spongiosis
- ▶ a mild lymphohistiocytic perivascular and interstitial papillary dermal infiltrate .
- ▶ There may be mild erythrocyte extravasation.

treatment

- ▶ **) Reassurance / Patient education:** Most patients require NO treatment
- ▶ **General measures:**
 - Wearing wide cotton underwear .
 - Cool baths with emollient soap .
- ▶ **Symptomatic treatment:** for pruritus
- ▶ **Sever PR:**
 - UVB
 - Systemic steroid
 - Acyclovir
 - erythromycin

2 - Ptyriasis rupra pilaris



Ptyriasis rupra pilaris



DD



Seborrheic dermatitis



Psoriasis

Clinical features

- ▶ Follicular papules with an erythematous base are a characteristic finding, including on the proximal dorsal fingers
- ▶
 - There is a coalescence of orange–red plaques, but with obvious islands of sparing (nutmeg grater)
- ▶
 - An orange–red waxy keratoderma of the palms and soles is often seen (sandle)
- ▶ Scaly scalp
- ▶ Thickend nail

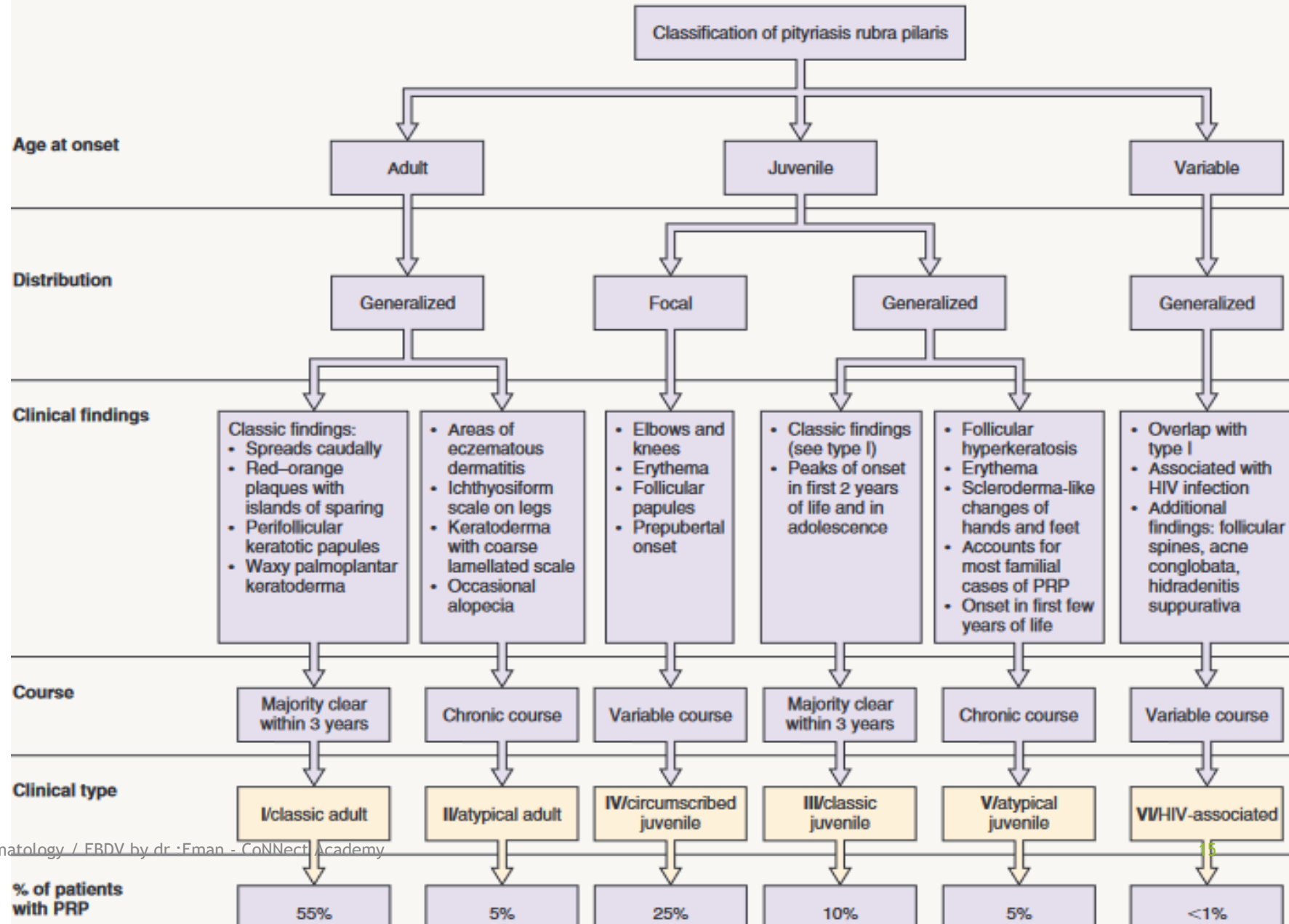
► **incidence:** PRP is a relatively rare disorder.

- Age: 2 age groups
 - o Adults: 4th - 6th decade
 - o Children: 1st - 2nd decade
- Sex: equal

► **Aetiopathogenesis :unkown**

- **Mutation in CARD 14 mutation**
- Vit A deficiency
- Disorder of keratinization
- May autoimmune

CLASSIFICATION OF PITYRIASIS RUBRA PILARIS



Atypical form



Histeopathology

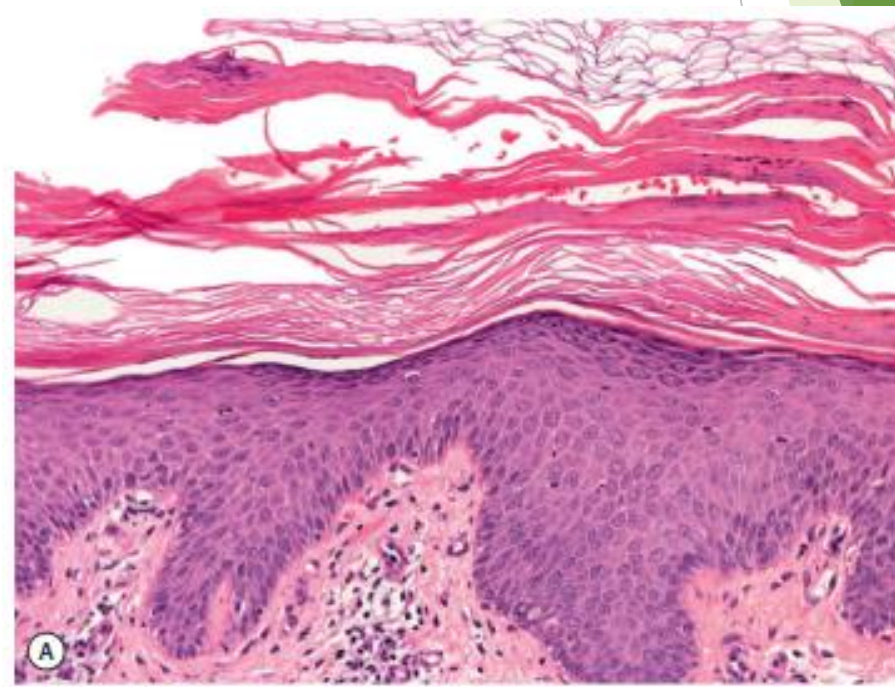
Follicular papule
dilated infundibula filled with
orthokeratotic plug.
• shoulders of parakeratosis
• Mild perifollicular
lymphocytic
infiltrate.

Interfollicular epidermis

Basket-weave hyperkeratosis
• Alternating vertical and
horizontal ortho- and para-
keratosis “**checkerboard**
pattern”.
• Focal or confluent
hypergranulosis
• Psoriasiform dermatitis: irregular
acanthosis with thick and short
rete'ridges



SCE-Dermatology / EBDV by dr.:Eman- CoNnect Academy



treatment

- ▶ Systemic
 - ▶ Isotretenoin : 1-1,5 mg /kg /day for 3 to 6 months
 - ▶ Acitretine
 - ▶ Methotrexat
- ▶ Topical treatment: retinoids, calcineurin inhibitors and vitamin D analogues.
- ▶ Phototherapy may aggravate

Pleva- plc



3- pleva , plc

- ▶ pityriasis lichenoides et varioliformis acuta (PLEVA) and pityriasis lichenoides chronica (PLC) are two ends of a disease spectrum
- ▶
 - Both entities are characterized by recurrent crops of spontaneously regressing erythematous papules
- ▶
 - Lesions of PLEVA are crusted and occasionally vesiculopustular, whereas in PLC they are scaly
- ▶
 - Individual patients may show intermediate or mixed lesions
- ▶
 - Histologically, an interface dermatitis with necrotic keratinocytes is seen
- ▶
 - The infiltrate is predominantly T cells that are often monoclonal

DD of pleva



An Bras Dermatol 10.1016/j.abd.2019.06.009



What is dd of plc ?

DD

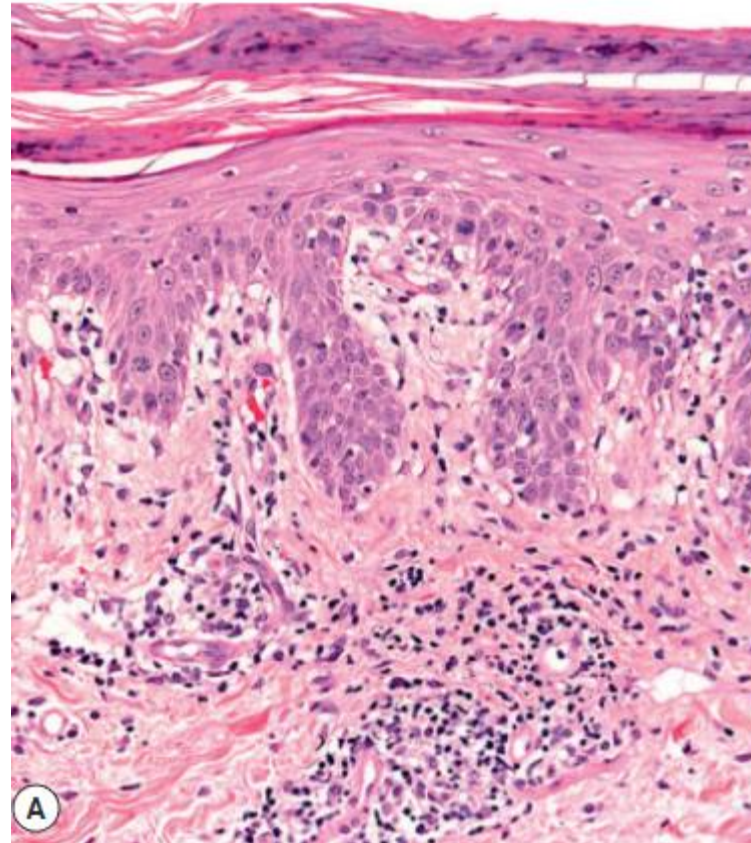
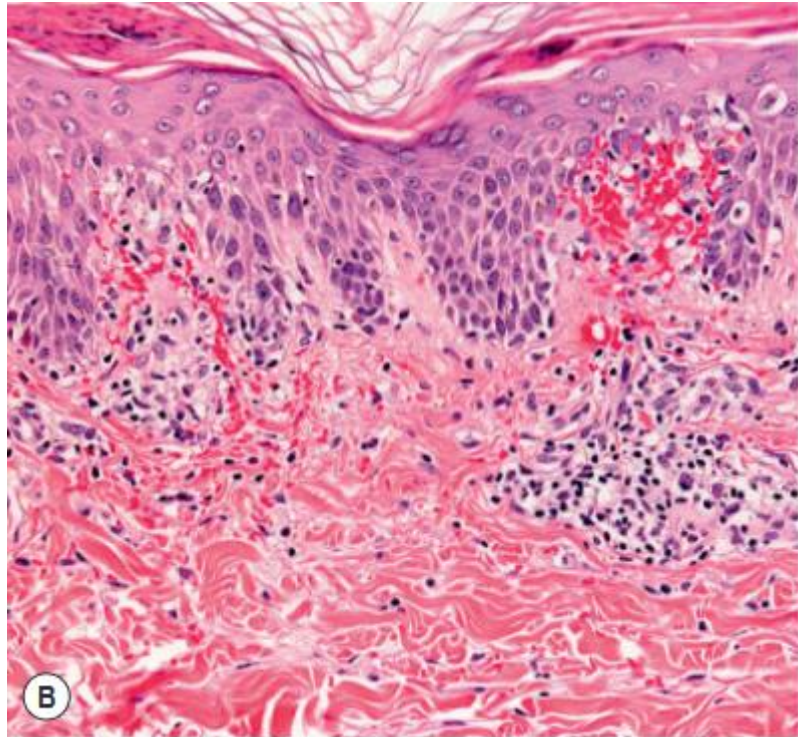
PRINCIPAL DIFFERENTIAL DIAGNOSIS OF PITYRIASIS LICHENOIDES ET VARIOLIFORMIS ACUTA (PLEVA) AND PITYRIASIS LICHENOIDES CHRONICA (PLC)	
PLEVA	PLC
Lymphomatoid papulosis	Small plaque parapsoriasis
Cutaneous small vessel vasculitis	Guttate psoriasis
Varicella, enteroviral exanthems	Lichen planus (exanthematous)
Arthropod reactions	Pityriasis rosea
Erythema multiforme	Secondary syphilis
Lichenoid drug eruption	Lymphomatoid papulosis
Folliculitis	Papular dermatitis
Dermatitis herpetiformis	Lichenoid drug eruption
	When hypopigmented, extrafacial pityriasis alba

- pityriasis lichenoides is a clonal T-cell lymphoproliferative disorder that exists as a disease spectrum with two ends.

	pleva	plc
Epidemiology	more common in children/young adults with male predominance.	
Aetiopathogenesis	the etiology is unknown. It may represent an immunological response to foreign antigens e.g. infectious agents (HIV, other viruses, streptococci, staphylococci..) or drugs(estrogen , progesterone , TNFinhibitor	
c/p	erythematous papules → vesicles → pustules → crusts and ulcers “polymorphic”.	small, firm lichenoid papules, with adherent scale “mica-like” that can be detached by gentle scraping.
age	most common form in children.	adult
symptoms	usually asymptomatic ± irritation / burning sensation.	

	pleva	plc
site	mainly trunk, thighs and upper arms (may be generalized)	
	Spontaneously resolves within weeks - months.	
healing	varioliform scars.	± post-inflammatory hyper- or hypo-pigmentation.
histopathology	Superficial perivascular lymphocytic infiltrate + vacular alteration	
	Focal parakeratosis. • Extensive epidermal damage. • Extensive vacuolar alteration. • More dense lymphocytic infiltrate. • Marked extra-vascularization of RBCs, papillary dermal edema and endothelial swelling. • Immunohistochemistry: CD8+ cells	Confluent (wide) parakeratosis. • Few necrotic keratinocytes • Acanthosis. • Subtle vacuolar alteration. • Milder lymphocytic infiltrate • Small number of RBCs extravasation. • Immunohistochemistry: CD4+ cells
	spontaneous resolution within weeks to months. Acute recurrences over a period of years, or chronicity may occur.	

	pleva	plc
treatment	Symptomatic treatment: antihistamines (for pruritus). • First-line therapy: topical corticosteroids or tacrolimus, oral tetracycline / erythromycin and phototherapy (natural sunlight, UVB, UVA1 or PUVA) • Second-line therapy: systemic corticosteroids, low-dose weekly methotrexate or other immunosuppressives e.g., IVIg, cyclosporine or etanercept	



**THERAPEUTIC LADDER FOR PITYRIASIS LICHENOIDES ET
VARIOLIFORMIS ACUTA (PLEVA) AND PITYRIASIS
LICHENOIDES CHRONICA (PLC)**



Topical corticosteroids (2)
Topical coal tar products (3)
Oral erythromycin (2)
Oral tetracyclines (2)
Oral azithromycin (3)
Sunlight (2)
Ultraviolet B (UVB) phototherapy – broadband or narrowband (2)
PUVA (psoralen plus UVA) phototherapy (2)
Methotrexate (2)
Cyclosporine (3)
Special treatments (3):
 Antihistamines*
 Systemic antibiotics†
 Systemic corticosteroids‡

*If there is pruritus.

†If there is secondary infection.

‡If there is fever, arthritis, or other systemic findings.

What is musha habermann disease



Febrile ulceronecrotic Mucha-Habermann disease (FUMHD): a severe variant of PLEVA associated with malaise, fever, arthritis, lymphadenopathy and/or bacteremia. Mucosal, gastrointestinal and pulmonary involvement may occur.

Granular parakeratosis



- ▶ Keratotic brownish red plaque in axilla mainly
- ▶ Common in female
- ▶ Cause :personal hyagen product , disorder of keratinization
- ▶ DD :other cause of intertrigo
- ▶ Tt :topical steroid , retinoid , systemic retenoid , antifungal

Ptyriasis rotunda



Actas Dermosifiliogr. 2016;107:535-7

- ▶ large circular and polycyclic scaly hyperpigmented patches with sharp margins .
- ▶ trunk and extremities.
- ▶ Like ichthyosis vulgaris
- ▶ Associated with chronic disorders:
 - ▶ Malnutrition
 - ▶ Malignancy
 - ▶ Tb , leprocy

TT: difficult to treat

Emolient , keratolytic

